

Nutrition Assessment

Policy: Staff must conduct a thorough nutritional assessment, which includes completing a Nutrition Interview. A nutrition interview must be completed at certification, recertification and mid-certification appointments. All questions within the Nutrition Interview must be asked and documented. The exceptions to this include:

- The anthropometrics fields for the biological mother and father.
- Local agencies that need to suspend referrals to the Breastfeeding Peer Counseling (BFPC) program for caseload management purposes may skip the BFPC support question.

Many questions lead to assignment of risk factors. Breastfeeding information within the interview may be updated as needed at follow-up appointments.

Note: Some questions in the Nutrition Interview are in bold font, this indicates questions used for data surveillance.

Regulation: CRF 246.4(a)(19)

Procedure/Additional Guidance:

A thorough WIC nutrition assessment evaluates the following areas:

- Growth
- Iron/Lead Screening
- Health/Medical
- Nutrition Practices
- Lifestyle
- Personal Factors

Growth: Growth is assessed by taking anthropometric measures on a regular basis and tracking growth patterns. Measures used to assess growth include: stature/weight, BMI, and prenatal weight gain. Growth is an important indicator of nutritional status. For example, poor growth may reflect inadequate nutritional intake or signal an underlying medical problem. Overweight can be associated with current or future health problems in both children and adults. Deviations from normal growth should be referred for follow up since early identification and intervention can prevent more serious health outcomes.

Iron/Lead Screening: Iron levels are measured regularly by performing either a hemoglobin or hematocrit test. The purpose of iron screening is to identify and prevent anemia (low iron in the blood). Lead levels for children are recorded from screenings done outside of WIC. Elevated blood lead levels can adversely affect behavior, development, and IQ. If a child has not had a lead testing, they must be referred to programs where they can obtain such a test.

Health/Medical: Health and Medical history and current conditions are assessed using the Nutrition Interview. Examples of medical conditions that have nutrition implications are: diseases (such as PKU, diabetes, high blood pressure), food allergies, and physical and mental disabilities. Additionally, medications can affect appetite. High-risk medical conditions require a follow up appointment with the high risk counselor at each certification period within 30 days of high risk identification.

Nutrition Practices: The WIC nutrition practices assessment is conducted through dialogue with the client about breastfeeding, if applicable, the foods they usually eat and their regular eating patterns. Using a discussion format, the WIC educator begins the interview with open-ended question asking the client to describe their typical eating and drinking habits. The educator evaluates the client's response against professionally recognized guidelines (e.g. Dietary Guidelines for Americans, My Plate Food Guide, and American Academy of Pediatrics) and the following criteria that are associated with a healthy diet:

- A variety of foods derived from different food groups
- Few processed foods in the diet
- Majority of foods consumed are nutrient dense (they contain a lot of vitamins and minerals)
- The amount of food eaten is appropriate

- Good eating habits (this includes having family meals, eating at set times rather than grazing, children not drinking from the bottle, etc.)

Then, based on the client's responses, the WIC staff person asks further questions to obtain additional information and/or clarify what has been said.

Breastfeeding complications and the breastfeeding description are included on the Infant Nutrition Interview under Nutrition Practices. The following table identifies an infant or child's breastfeeding status:

Description	Definition
Exclusively Breastfeeding	Has been fed only human milk, vitamins, minerals, and/or medications. (This is equivalent to the CDC definition of exclusive)
Primarily Exclusive/ No Formula Package	Has been fed something other than human milk, vitamins, minerals and/or medications on rare occasion or one time feeding of infant formula, human milk fortifier, cow's milk, juice, sugar water, rehydration solution, water, baby food, or anything else. This identifies an infant whose exclusive breastfeeding is interrupted because of special circumstances (e.g., acute illness, hospitalization, caregiver misinformation) is fed.
Primarily Exclusive/ Complementary Foods	Has been fed any complementary foods (e.g. cereal, baby food, table foods) in addition to only being fed human milk, vitamins, minerals and/or medications. These foods are provided on a routine or ongoing basis regardless of the amount.
Partially Breastfeeding	Breastfeeding and receiving formula (WIC or non WIC supplied formula). May also be fed complementary foods.
Breastfeeding Child	Greater than or equal to 12 months and continuing breastfeeding.
No Longer Breastfeeding	Was breastfeeding at some point in time but has now discontinued.
Never Breastfed	Was never breastfed.

Lifestyle: Assessment of lifestyle involves finding out about physical activity (including the types and amounts of physical activity they get) and substance use (such as abuse or exposure to alcohol, drugs, cigarettes or second hand smoke).

Immunizations: Immunization information is captured during the infant and child interview for DTaP shot records only.

Oral Health: Oral health information is captured during the infant and child interview to obtain information regarding dental and other oral health concerns.

Breastfeeding Support: For all breastfeeding women, breastfeeding support is included as part of the assessment during the nutrition interview. Breastfeeding complications and assignment of the BFPC are captured on this panel.

Social Environment: Social Environment includes: culture, family structure, education, living and work situations, abuse/neglect, family planning and primary caregiver with limited ability to make appropriate feeding decisions. Understanding social environment enables WIC staff to provide client centered care and individualized referrals and counseling.